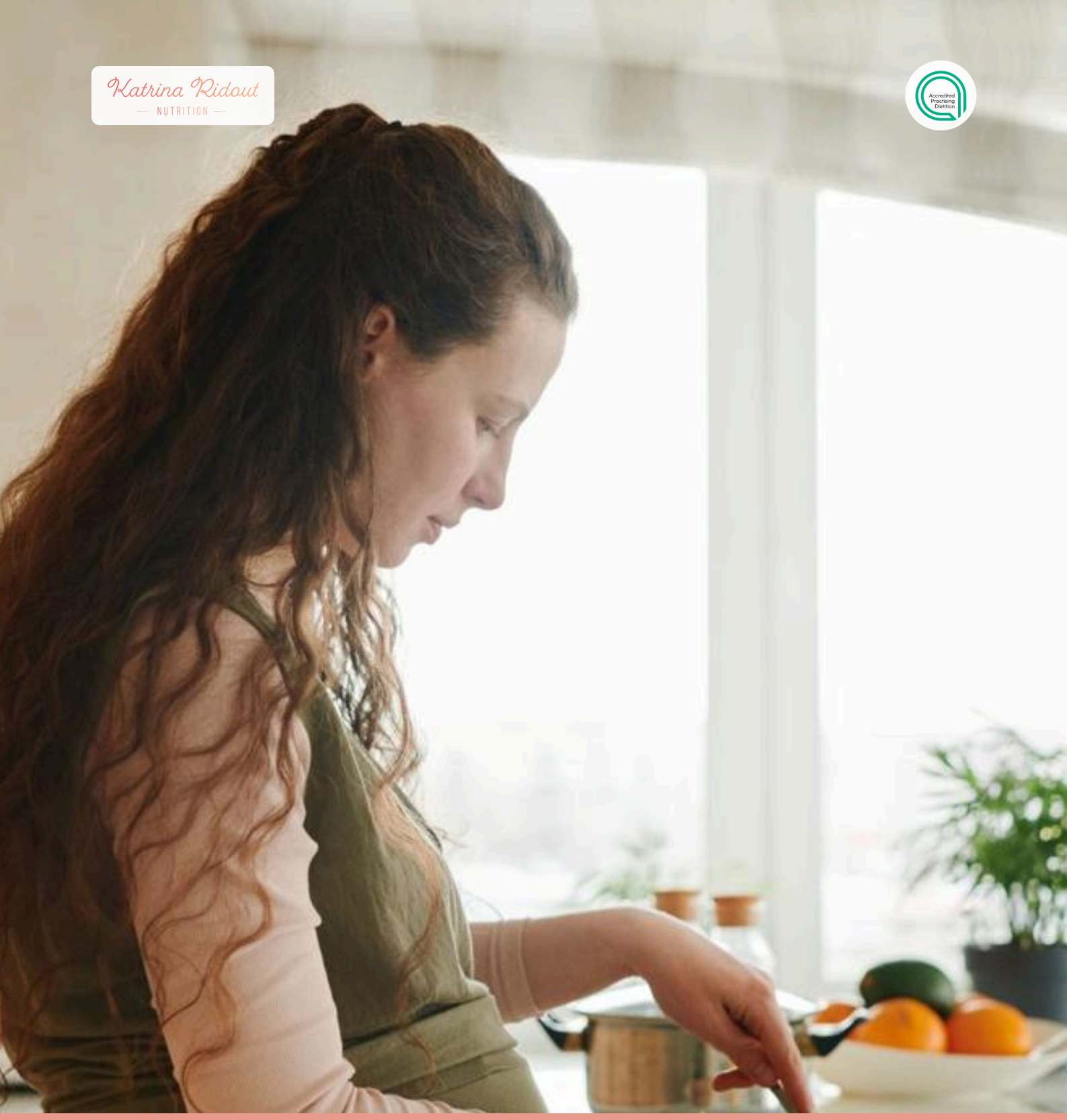
Katrina Ridout
NUTRITION

Accredited Practising Dietitian



PLANNING A PREGNANCY

PRECONCEPTION CHECKLIST

WHERE TO START IN THE THREE MONTHS BEFORE TRYING



WELCOME

You asked for my preconception checklist, so here it is. Three short parts: the checks worth asking your GP about, the two supplements with clear Australian guidance, and the first food change I would make before anything else.

Eggs and sperm take about three months to mature. That is your runway. This checklist gets you to the starting line. What happens after that depends on your results, your cycle and your history, and that is the part I do with you.

Kat Ridout
Accredited Practising Dietitian
Katrina Ridout Nutrition

Kat x

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PART 1

Tests to ask your GP about

Book a GP visit around three months before you start trying. That gives time to fix anything that turns up. Your GP decides which tests are right for you; this list is a starting point for the conversation.

Iron studies (ferritin)

Needed for ovulation and for the extra blood volume of pregnancy. Low iron is common.

Vitamin D

Supports hormone regulation and egg quality. Many Australians are low.

Thyroid function (TSH)

Your thyroid works about 50 per cent harder in pregnancy. A thyroid problem changes the supplement advice, so it is worth knowing first.

Blood glucose or HbA1c

Worth asking about if you have PCOS, a family history of diabetes, or gestational diabetes in a past pregnancy.

Rubella and varicella immunity

Some vaccines cannot be given in pregnancy, so immunity is checked beforehand.

Cervical screening

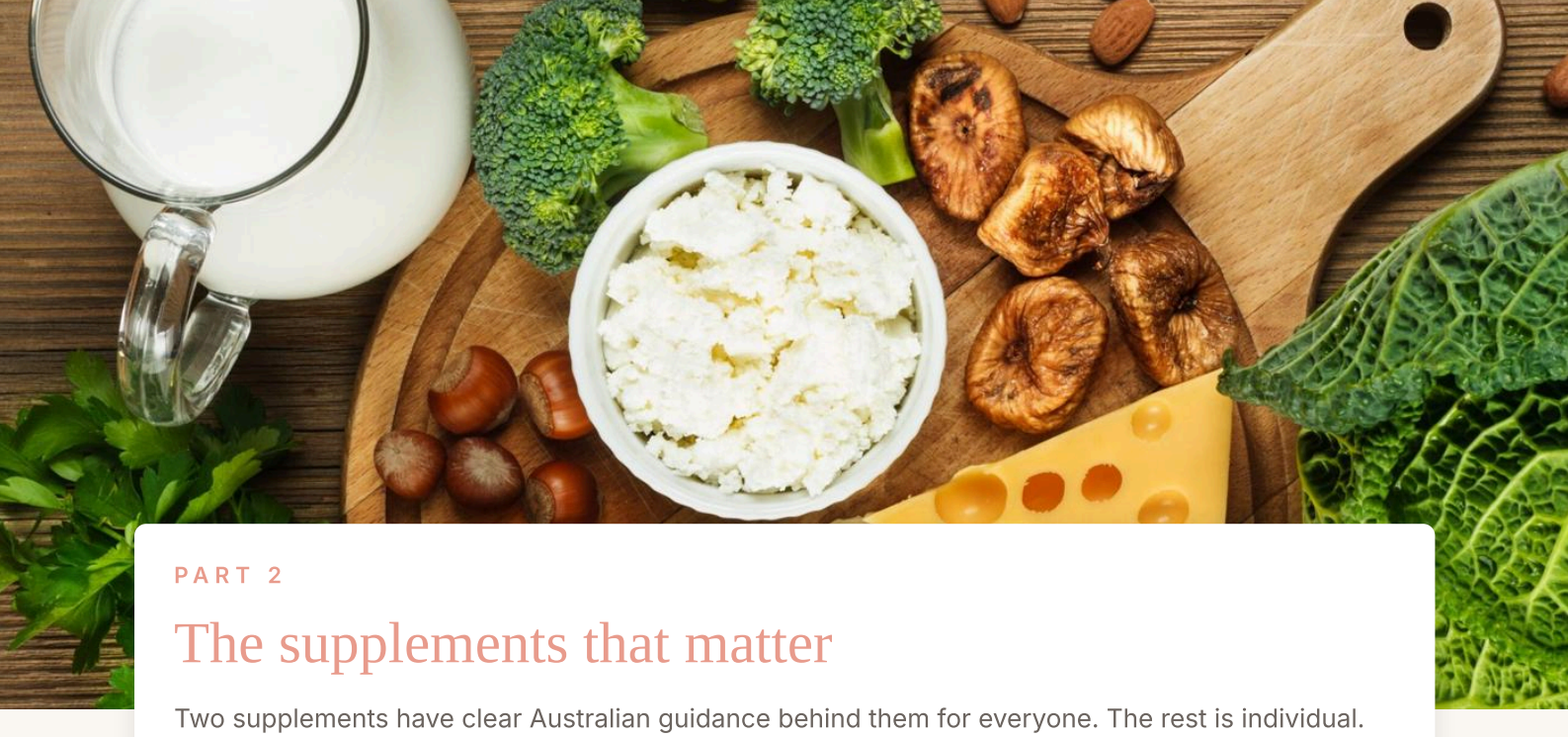
Check it is up to date so it does not fall due mid-pregnancy.

Your medicines and supplements

Bring every bottle, front and back. Some doses need adjusting before conception.

Then bring the results to me

A number on a page is not a plan. What your ferritin, vitamin D and thyroid results mean for your food, your



PART 2

The supplements that matter

Two supplements have clear Australian guidance behind them for everyone. The rest is individual.

Folic acid: 400 micrograms a day

Start at least one month before conception and continue through the first trimester. The neural tube closes before most women know they are pregnant, which is why starting early matters.

Some women need a higher dose: a personal or family history of neural tube defects, certain conditions or medicines. Your GP or specialist decides that, not the label.

Iodine: 150 micrograms a day

Recommended by the NHMRC when planning, pregnant and breastfeeding. Most of us do not get enough from food.

More is not better. There is no benefit above 150 micrograms from a supplement. Avoid kelp and seaweed supplements. If you have a thyroid condition, talk to your doctor first.

Check your label

Turn your pregnancy multivitamin over. It should list folic acid at 400 micrograms and iodine at around 150 micrograms. Some products contain much more folic acid than you need, and it is easy to double up if you take more than one product.

Everything else depends on you

Vitamin D, iron, omega-3, choline, inositol: whether you need them, how much, and which product, depends on your bloods, your cycle and your history. In your initial consultation you get your individualised supplement plan from day one, checked against what you are already taking.

PART 3

The first food change

Not a diet. One change you can start this week, and the two that come next.

1 Eat eggs most days

One large egg gives roughly 125 to 150 mg of choline, almost all of it in the yolk. The pregnancy target is 440 mg a day and only about one in four Australian women measured reach it. Two eggs gets you a long way there before anything else on the plate is counted.

2 Your iron plan

Which iron-rich foods, how often, and what to pair them with depends on your ferritin result and whether you eat meat. This is the first thing we set up once your bloods are back.

3 Your carbohydrate and cycle plan

How you spread carbohydrate across the day affects insulin, ovulation and energy, and the right pattern is different for PCOS, endometriosis, thyroid conditions and a regular cycle. This is the core of your written plan.

And for your partner

Sperm quality responds to diet over the same three-month window. The couples initial consultation covers you both in one 90-minute session.







PART 4

Where a checklist stops and a plan starts

A checklist is the same for everyone. A plan is built on your results, your cycle and your history, and it changes as you move from trying, through IVF or pregnancy, and into the first year with your baby.

This checklist gives you

- [ ] The tests to ask your GP about
- [ ] The two supplements everyone needs
- [ ] The first food change to make
- [ ] A three-month head start

A consult with Kat gives you

- Your results read and explained
- Your supplements reviewed, doses set for you
- A written plan within two business days
- Support that moves with you through IVF, pregnancy and postpartum

Three ways to start

Free 15-minute call

$0 · phone
Not sure where to start? We talk through what is going on and whether I can help. No advice on the call, no pressure.

One-Question Consult

$129 · 30 min video
New clients only. One question answered properly, with a one-page written plan. Credited to a full consult within 30 days.

Initial consultation

$295 · 75 min
Full assessment, your individualised supplement plan from day one, written plan and resources within two business days. Medicare rebate may apply with a GP Chronic Condition Management Plan.

Most clients then work with me over a program of five or seven consultations across their journey. I walk you through the options on the free call.





PART 5

YOUR NEXT STEP

You now know what to check and where to start. The next step is turning your results into a plan that is yours. A free 15-minute call is the easiest way to begin. No advice and no pitch on the call, just whether I can help and which option fits.

I see women in person at Milton Village Medical in Brisbane and by telehealth anywhere in Australia. A Medicare rebate may apply with a GP Chronic Condition Management Plan.

Book a free 15-minute call

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General information for people planning a pregnancy in Australia, not individual advice. Talk to your GP, obstetrician or fertility specialist about your own situation. Sources: NHMRC, NSW Health, Australian and New Zealand Nutrient Reference Values, Barwon Infant Study.